lipoprotein to borderline-high was seen with both baricitinib doses but the increases to high or very-high LDL were similar between baricitinib and placebo, and no major adverse cardiovascular events was reported.

Currently approved systemic therapies for AD patients who fail topical therapies are limited. Systemic corticosteroids, although FDA-approved and used for short-term improvement, are not recommended by AD treatment guidelines because of short-term and long-term AEs, resulting in an overall unfavorable risk-benefit profile.[16] Conventional immunosuppressants (eg, methotrexate, cyclosporine, mycophenolate mofetil, and azathioprine) are used for severe AD. Because of their significant AE risk, AD guidelines recommend only short-term treatment durations (6-12 months).[16] Baricitinib can be an oral treatment option for short-interval use in patients with more moderate, flaring disease and for chronic, long-term use, with growing long-term safety data available from AD and RA.[13,15]

BREEZE-AD5 enrolled a racially diverse patient population. Emerging research suggests race is a factor in the phenotypic expression of AD. Th2 axis activation is exhibited in European, African, and Asian Americans with AD, yet variations in TH17/TH22/TH1 pathways are specific to the different endophenotypes.[3,6,17] The study data, therefore, may be generalizable to the diverse North American population.

Data presented here are from the first 16-week treatment period. BREEZE-AD5 is limited by the baricitinib doses evaluated; only 2-mg and 1-mg doses were studied because baricitinib 2 mg is approved for RA in North America.[18] In prior trials, 2-mg and 4-mg doses were studied.[4]

CONCLUSIONS

BREEZE-AD5 is the fifth positive phase 3 baricitinib AD trial. When treated with monotherapy baricitinib 2 mg once daily, adults with moderate-to-severe AD achieved statistically significant improvements over placebo in week-16 EASI75 responses. Patients treated with baricitinib reported improvements with other secondary efficacy endpoints. No new safety concerns were reported.

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Conflicts of interest

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